

EYE/VISION SYMPTOMS

Give urgent attention to the patient with eye or vision symptoms and any of:

- New sudden asymmetric weakness or numbness of face, arm or leg; difficulty speaking or visual disturbance: consider stroke or TIA →136.
- BP ≥ 180/110 and not pregnant →132.
- Pregnant or up to 1 week post-partum, and BP ≥ 140/90: treat as **severe pre-eclampsia** →159.
- Yellow eyes: **jaundice** likely →79.
- Whole eyelid swollen, red and painful: **orbital cellulitis** likely
- One painful red eye
- Sudden loss or change in vision (including blurred or reduced vision)
- Shingles involving eye or nose
- Penetrating injury
- Eyelid laceration
- Penetrating or metallic foreign body
- Chemical burn
- Corneal ulcer
- Hazy cornea
- Sudden drooping of eyelid

Manage and refer urgently:

- If painful eye with redness, blurred vision, haloes around light, dilated unreactive pupil, headache or nausea/vomiting, **acute glaucoma** likely. Give **acetazolamide** orally 500mg, then 250mg 6 hourly.
- If **orbital cellulitis** likely, give **ceftriaxone** 2g IV¹/IM. Avoid injecting > 1g IM at one injection site.
- If **chemical burn**: irrigate eye for at least 20 minutes with **sodium chloride 0.9%** or water. If pain, instil 1 drop **tetracaine 1%** eye drops to affected eye. Apply **chloramphenicol 1%** ointment 6 hourly.
- If **penetrating or metallic foreign body**: avoid removing. Cover gently. Avoid lying flat. If deep corneal/scleral injury and delay in transfer, instil 1 drop **atropine 1%** and **chloramphenicol 1%** ointment.
- If pain, give **paracetamol** 1g 4 hourly.

Approach to patient with eye/vision symptoms not needing urgent attention

Eyes discharging or watery. Is there a prominent itch?		Red or swollen eyelid/s	Superficial foreign body (FB)	Poor vision
Yes	No			
	Is the discharge clear or pus?			
	Clear			
	Pus			
- If both eyes involved or patient has eczema, hayfever or asthma, treat for likely allergic conjunctivitis: - Help to identify and advise to avoid triggers². - Give oxymetazoline 0.025% eye drops 1-2 drops in each eye 6 hourly up to 7 days and advise to apply cold compresses. If no better after 7 days: give instead anti-allergy eye drops (e.g. olopatidine 0.1% 1 drop 12 hourly) for 1-3 months or long-term. - If symptoms > 1 month, add cetirizine 10mg once daily until itch controlled. - If recurrent nose problem, exclude allergic rhinitis ↪ 34. If recurrent skin problem, exclude urticaria and eczema ↪ 67. If recurrent cough or wheeze, exclude asthma ↪ 112. - If one eye involved and no eczema, hayfever or asthma, localised cause likely: wash eye with clean water and try to identify and remove cause. Give oxymetazoline 0.025% eye drops 1-2 drops 6 hourly for 3 days. If no better after 24 hours, advise to return: refer.	Viral conjunctivitis likely - Apply cold compresses. - Give oxymetazoline 0.025% eye drops 1-2 drops 6 hourly up to 7 days. - Infectious: only return to work once better/no discharge. Bacterial conjunctivitis likely - Wipe eyes gently from inside to outside with clean cotton wool soaked in sodium chloride 0.9% until pus clears. - Give chloramphenicol 1% ointment 6 hourly in each eye for 7 days. - Return to work after 2 days of treatment and no pus.	- Wash lid/s twice a day with warm water. - Give chloramphenicol 1% ointment 6 hourly for 7 days. - If yellow lump on eyelid, apply frequent warm compresses. - Refer to eye OPD if: - Lump no better with warm compresses - Eyelashes touching cornea - Eyelids bent in/out.	- Wash out eye with clean water or sodium chloride 0.9%. - If FB not visible, use fluorescein stain and ultraviolet light. - Instil 1 drop tetracaine 1% eye drops³ and gently remove FB with moist cotton bud. - If under eyelid, pull top eyelid over bottom eyelid and release. - Apply eye shield until tetracaine has worn off. - Refer same day if: - Removal unsuccessful - Damage to eye - Abnormal vision or eye movement - No better 24 hours after removal	- Check vision using Snellen E chart and pinhole test: - If vision improves when looking through pinhole and service available, refer for glasses. - If vision no better with pinhole, service not available or unsure, refer for full assessment. - Exclude diabetes ↪ 17 and hypertension ↪ 132. - Test for HIV ↪ 110.

¹Do not mix Ringer's lactate and IV ceftriaxone. Flush IV line with **sodium chloride 0.9%** before and after IV ceftriaxone. ²Common triggers include pollens, household pets, house dust mite, cockroaches and moulds. ³Strictly avoid giving tetracaine eye drops to patient to take home as they can cause blindness if used too often.